

***Technical
Writing***



Technical Writing

Lecture (4)

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2025 - 2026

Nursing Report Sheet Template

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PATIENT REPORT SHEET TEMPLATE

Patient Name		DOB	Room/Bed	Date/Shift: AM/ PM/ Night
Code	Allergies			Isolation
Diagnosis				
PMH			PSH	
Meds			IV Access ☐ PIV ☐ PICC ☐ Central Line ☐ Port Location: _____	
Last Set of Vitals: T: ____ HR: ____ BP: ____ RR: ____ SpO2: ____ %				
Pain Level: ____/10 Location: _____ Management: _____				
Neuro: ☐ Alert & Oriented x ____ ☐ Confused ☐ Lethargic ☐ Unresponsive Cardiac: ☐ Regular ☐ Irregular ☐ Murmur ☐ Edema ____+ Respiratory: ☐ Normal ☐ Labored ☐ Oxygen ____L via ____ GI: ☐ Normal ☐ NPO ☐ NG/OG Tube ☐ Bowel Sounds ____+ GU: ☐ Voiding ☐ Foley ☐ Incontinence ☐ Dialysis Skin: ☐ Intact ☐ Wounds: _____				
Pending procedures		Recent labs/imaging		Results of note
Physician updates		Meds due		
Treatments/Nursing tasks		Patient/family concerns	To do	
Current care plan		Discharge plan		

Tips for Filling Out a Nursing Report Sheet Efficiently

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- 1- Use Abbreviations & Symbols.
- 2- Focus on What's Important.
- 3- Keep It Legible & Organized.
- 4- Update as You Go.
- 5- Complete the sheet comprehensively and accurately.
- 6- Use clear and concise language.
- 7- Document all relevant observations and concerns.
- 8- Proofread the sheet carefully before handing off to the next nurse.
- 9- Ask questions and clarify any uncertainties.
- 10- Utilize standardized abbreviations and terminology..

Tips for Filling Out a Nursing Report Sheet Efficiently

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By mastering nursing report sheets, you can:

- ✓ Enhance communication,
- ✓ Improve patient care,
- ✓ Ensure a smooth and safe transition between shifts.

Remember,

Accurate and efficient reporting is not just a good practice,
it's a vital aspect of providing quality patient care.

Case study # 01

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Sarah Johnson, a 37-year-old woman, was admitted to room 204B during the morning shift on October 17, 2025. She has a medical history of asthma and seasonal allergies and previously had an appendectomy in 2010.

Sarah was diagnosed with community-acquired pneumonia. Her code status is G312, and she has no isolation precautions. She is allergic to penicillin.

At the time of assessment, her temperature was 38.2°C, heart rate 96 bpm, blood pressure 118/74 mmHg, respiratory rate 20 breaths per minute, and oxygen saturation 94% on room air. She reported mild chest pain (3 out of 10), which was managed with Tylenol.

A peripheral IV line was placed in her left forearm. Her breathing was labored, so she was placed on 2 liters of oxygen via nasal cannula. Her cardiac rhythm was regular, bowel and bladder function were normal, and her skin was intact..

Case study # 01 (Cont.)

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Her current medications include Azithromycin 500 mg orally once daily, Albuterol inhaler as needed, and Acetaminophen 650 mg every 6 hours as needed. A chest X-ray revealed a left lower lobe infiltrate, and her white blood cell count was elevated ($13.5 \times 10^9/L$). The physician instructed to continue antibiotics and monitor her temperature every four hours. Sarah was encouraged to ambulate as tolerated and perform deep breathing exercises. The care plan focuses on treating the infection, managing fever, and promoting lung expansion. She will be discharged home once she is afebrile and able to take oral medications..

Nurses Report Sheet

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Sarah Johnson, a 37-year-old woman, was admitted to room 204B during the morning shift on October 17, 2025. She has a medical history of asthma and seasonal allergies and previously had an appendectomy in 2010.

Sarah was diagnosed with community-acquired pneumonia. Her code status is G312, and she has no isolation precautions. She is allergic to penicillin.

Patient Name		DOB	Room/Bed	Date/Shift: AM/ PM/ Night
Code	Allergies			Isolation
Diagnosis				

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Sarah was diagnosed with **community-acquired pneumonia**. Her code status is **G312**, and she has **no** isolation precautions. She is allergic to **penicillin**.

Patient Name	Mrs. Sarah Johnson	DOB	37 Y	Room/Bed	204/B	Date/Shift	AM/ 17/10/2025
Code	G312	Allergies	penicillin			Isolation	No
Diagnosis	community-acquired pneumonia						

Nurses Report Sheet

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Sarah Johnson, a 37-year-old woman, was admitted to room 204B during the morning shift on October 17, 2025. She has a medical history of asthma and seasonal allergies and previously had an appendectomy in 2010.

Sarah was diagnosed with community-acquired pneumonia. Her code status is G312, and she has no isolation precautions. She is allergic to penicillin.

PMH	PSH
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Nurses Report Sheet

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~~Sarah Johnson, a 37-year-old woman, was admitted to room 204B during the morning shift on October 17, 2025. She has a medical history of **asthma and seasonal allergies** and previously had an **appendectomy in 2010**.~~

~~Sarah was diagnosed with community-acquired pneumonia. Her code status is G312, and she has no isolation precautions. She is allergic to penicillin.~~

PMH	PSH
- Asthma - Seasonal allergies	Appendectomy (2010)

Nurses Report Sheet

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At the time of assessment, her temperature was 38.2°C, heart rate 96 bpm, blood pressure 118/74 mmHg, respiratory rate 20 breaths per minute, and oxygen saturation 94% on room air.

She reported mild chest pain (3 out of 10), which was managed with Tylenol.

Last Set of Vitals: T: ____ HR: ____ BP: ____ RR: ____ SpO2: ____%
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Pain Level: ____/10 Location: _____ Management: _____

Nurses Report Sheet

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At the time of assessment, her temperature was **38.2°C**, heart rate **96** bpm, blood pressure **118/74** mmHg, respiratory rate **20** breaths per minute, and oxygen saturation **94%** on room air.

She reported mild **chest** pain (**3 out of 10**), which was managed with **Tylenol**.

Last Set of Vitals: T: 38.2°C HR: 96 BP: 118/74 RR: 20 SpO2: 94 %
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Pain Level: 3 /10 Location: Chest Management: Tylenol
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Nurses Report Sheet

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A peripheral IV line was placed in her left forearm. Her breathing was labored, so she was placed on 2 liters of oxygen via nasal cannula. Her cardiac rhythm was regular, bowel and bladder function were normal, and her skin was intact. Her current medications include Azithromycin 500 mg orally once daily, Albuterol inhaler as needed, and Acetaminophen 650 mg every 6 hours as needed.

Meds	IV Access ☐ PIV ☐ PICC ☐ Central Line ☐ Port Location: _____
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Nurses Report Sheet

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A **peripheral IV line was placed in her left forearm.** ~~Her breathing was labored, so she was placed on 2 liters of oxygen via nasal cannula. Her cardiac rhythm was regular, bowel and bladder function were normal, and her skin was intact.~~

Her current medications include **Azithromycin 500 mg orally once daily, Albuterol inhaler as needed, and Acetaminophen 650 mg every 6 hours as needed.**

Meds - Azithromycin 500 mg PO daily, - Albuterol inhaler PRN (as needed), - Acetaminophen 650 mg PO Q6 PRN.	IV Access ☒ PIV left forearm ☐ PICC ☐ Central Line ☐ Port Location: _____
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Nurses Report Sheet

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A peripheral IV line was placed in her left forearm. Her breathing was labored, so she was placed on 2 liters of oxygen via nasal cannula. Her cardiac rhythm was regular, bowel and bladder function were normal, and her skin was intact. Her current medications include Azithromycin 500 mg orally once daily, Albuterol inhaler as needed, and Acetaminophen 650 mg every 6 hours as needed.

Neuro: ☐ Alert & Oriented x __ ☐ Confused ☐ Lethargic ☐ Unresponsive
Cardiac: ☐ Regular ☐ Irregular ☐ Murmur ☐ Edema __+
Respiratory: ☐ Normal ☐ Labored ☐ Oxygen __L via __
GI: ☐ Normal ☐ NPO ☐ NG/OG Tube ☐ Bowel Sounds __+
GU: ☐ Voiding ☐ Foley ☐ Incontinence ☐ Dialysis
Skin: ☐ Intact ☐ Wounds: _____

Nurses Report Sheet

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~~A peripheral IV line was placed in her left forearm. Her breathing was labored, so she was placed on 2 liters of oxygen via nasal cannula. Her cardiac rhythm was regular, bowel and bladder function were normal, and her skin was intact. Her current medications include Azithromycin 500 mg orally once daily, Albuterol inhaler as needed, and Acetaminophen 650 mg every 6 hours as needed.~~

Neuro:	☒ Alert & Oriented x __	☐ Confused	☐ Lethargic	☐ Unresponsive
Cardiac:	☒ Regular	☐ Irregular	☐ Murmur	☐ Edema __+
Respiratory:	☐ Normal	☒ Labored	☐ Oxygen	<u>2</u> L via NC (nasal cannula)
GI:	☒ Normal	☐ NPO	☐ NG/OG Tube	☐ Bowel Sounds __+
GU:	☒ Voiding	☐ Foley	☐ Incontinence	☐ Dialysis
Skin:	☒ Intact	☐ Wounds:	_____	

Nurses Report Sheet

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A chest X-ray revealed a left lower lobe infiltrate, and her white blood cell count was elevated ($13.5 \times 10^9/L$). The physician instructed to continue antibiotics and monitor her temperature every four hours.

Pending procedures	Recent labs/imaging	Results of note
Physician updates	Meds due	

Nurses Report Sheet

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A **chest X-ray** revealed a left lower lobe infiltrate, and her **white blood cell count was elevated ($13.5 \times 10^9/L$)**. The physician instructed to **continue antibiotics and monitor her temperature every four hours**.

Pending procedures chest X-ray follow up	Recent labs/imaging CBC, CMP CXR shows left lower lobe	Results of note elevated WBC ($13.5 \times 10^9/L$)
Physician updates - continue antibiotics - monitor her temperature Q4	infiltration Meds due Azithromycin 500 mg PO daily (from Meds)	

Nurses Report Sheet

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Sarah was encouraged to ambulate as tolerated and perform deep breathing exercises. The care plan focuses on treating the infection, managing fever, and promoting lung expansion. She will be discharged home once she is afebrile and able to take oral medications.

Treatments/Nursing tasks	Patient/family concerns	To do
Current care plan	Discharge plan	

Nurses Report Sheet

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Sarah was **encouraged to ambulate as tolerated and perform deep breathing exercises**. The care plan focuses on **treating the infection, managing fever, and promoting lung expansion**. She will be discharged **home once she is afebrile and able to take oral medications**.

Treatments/Nursing tasks - encouraged to ambulate as tolerated, - perform deep breathing exercises	Patient/family concerns - Fatigue, - Cough.	To do - Monitor oxygen. - Reassess pain. - Review labs.
Current care plan - treat the infection, - manage fever, - promote lung expansion.	Discharge plan home when afebrile and tolerating PO meds.	

Case study # 02

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Liam Carter, a 9-year-old boy, was admitted to the pediatric unit in room 112A during the evening shift on October 17, 2025. His diagnosis was Influenza B, and he was placed under droplet isolation precautions. Liam has a past medical history of mild intermittent asthma, but no history of surgeries. He has no known allergies.

At the time of assessment, Liam's temperature was 38.5°C, heart rate 110 bpm, blood pressure 100/60 mmHg, respiratory rate 22 breaths per minute, and oxygen saturation 97% on room air. He complained of a headache with a pain level of 4 out of 10, which was treated with Ibuprofen.

He had a peripheral IV line inserted in his right hand for medication administration. His neurological, cardiac, respiratory, gastrointestinal, and genitourinary assessments were all normal, and his skin was intact. A rapid flu test came back positive for Influenza B.

Liam's current medications included Oseltamivir (Tamiflu) 45 mg twice daily, Albuterol nebulizer every 6 hours as needed for wheezing, and Ibuprofen 10 mg/kg every 6 hours as needed for fever. The physician advised to continue Tamiflu and provide supportive care. Nursing interventions included encouraging fluids, monitoring temperature, and maintaining isolation precautions. Liam's mother was concerned about his decreased appetite. The nursing plan emphasized symptom control, hydration, and infection control. Liam was expected to be discharged once he was afebrile for 24 hours and eating well.

Assignment # 02

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Fill the given nursing report with the data shown in case study # 02

Due Date: 5/11/2025

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